

breathe out slowly. Repeat a few times.

Next, welcome your therapist parts—the ones who want new ideas for your clients. What do they want to learn about the addictive process? What are their strengths? What are their challenges? Are they preoccupied with any particular clients today? Do they hold any clients with addictive processes, even from the past, in your heart? Do they react to or find it hard to connect with any particular clients? Listen to these parts. When you're ready, write down their answers. How do you feel toward them now? Tell them you've been listening. Thank them for whatever they shared. Let them know they can stay in relationship with you.

Now, turn to any parts who focus on family members or loved ones who have struggled with addictive processes in the past or who struggle now. Notice the images or memories they hold. If you'd like, write down the name of each family member or loved one. What do your parts want you to know about this person? How have they been affected by this person? Has your connection with this person changed at all (that is, gotten better or worse)? If so, what happened?

Take time to record what your parts are holding for each of the people mentioned. How are these parts doing now? Listen carefully. Do they notice you? What do they want you to learn from this manual? Thank them for whatever they shared. Let them know they can stay in relationship with you.

Finally, when you are ready, invite any parts who promote some type of addictive process to serve you. What do you notice about them? How do you feel toward them? Do you notice any other parts reacting toward them? If so, ask the reactive ones to step back and trust you. How have they tried to help? What do the addictive parts need from you today? What do they want you to understand or learn from this manual? Take time to record their answers and thank them for whatever they shared. Let them know they can stay in relationship with you.

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OUR AGENDA

In IFS, we don't come at protectors in an addictive system with our own change agenda. This stance may feel counterintuitive or even downright unethical for a couple of reasons. First, people whose addictive parts cause dysfunction for themselves are bound to annoy and worry both themselves and others. Second, therapists are trained (and thereafter continually urged) to pursue change in their clients. But with extreme protectors, it's most effective to think systemically, be optimistic, and have no agenda other than getting them connected to the Self and letting them explain their positive intentions. For this to happen, our own managers have to relax. We must be able help our clients be curious and compassionate by being curious and compassionate ourselves. This is what we call being Self-led.

What we know can help us be and remain Self-led throughout therapy. We know that exiled emotions and memories are connected to the client's addictive process. And once we get to know a client, we understand why their protectors deploy when they do, even if the client doesn't recognize the association at first. We also know that inner critics work hard in an addictive system, so we get curious about parts who are critical of other parts or who blame other people. To help clients make this connection, we invite them to slow down, turn their attention inside, notice what happened in the days or hours before they gave into an addictive urge, and notice what they feel right now. We know their needy parts feel desperate and abandoned. And we know that internal matters must improve before the client will try any new behaviors. Therefore, we do therapy with a system, not a symptom.

THE ADDICTION POLARITY

As you treat clients who are engaged in addictive processes, keep the following information front and center in your mind. Polarized protector teams are the norm in addictive processes—and these teams have very different perspectives. For managers, engaging in the compulsive behavior is like volunteering to drown. For firefighters, abstinence is like throwing your life jacket away as the waters are rising. As a result, it's good to consider whom you're talking with. If you're only hearing from one side, you can be assured that the other side exists and will take the floor at some point. To avoid reinforcing polarized conflicts, it's best to just assume that the client has long been bombarded with differing views about using, stopping using, and even showing up for an assessment interview or first session—and avoid taking a side.

If your head is spinning because the client is being obviously contradictory—for example, saying, “I have to stop drinking!” and “Why is everyone so uptight about what I do after work?”—that means you're listening to an argument between the two teams in real time. In this case, your best option is to call the game: “I hear a part who wants to stop drinking... Now I hear a part who doesn't want to stop drinking.” Combine your nonjudgmental interest with a light, polite tone that will signal to firefighters—the ones who are hardest to engage—that you might confound their negative expectations.

If the client starts therapy by complaining about having to be in therapy, someone in their life probably pushed them to get treatment and you're listening to a firefighter complain (as we illustrated in the example of Roger,